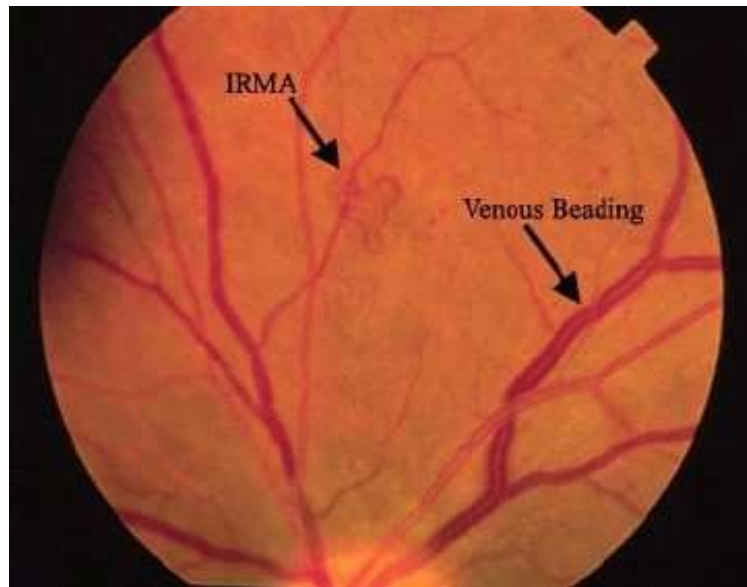


NPDR: grade the ischaemic warning signs

Clinical M3-M5 | Focused Type B notes

Intraretinal changes are not surface proliferation

Venous beading and IRMA



The original arrows identify IRMA and venous beading. These warn of retinal ischaemia; neither finding alone is a surface neovascular network. Supplied M3 Anki, media 3.

Grade with a complete fundus survey

Grade	Decisive findings
Mild NPDR	Microaneurysms only.
Moderate NPDR	More than microaneurysms, but less than the severe threshold.
Severe NPDR	Any 4-2-1 criterion, and no PDR.
PDR	New vessels at the disc or elsewhere; may bleed into preretinal/vitreous spaces.

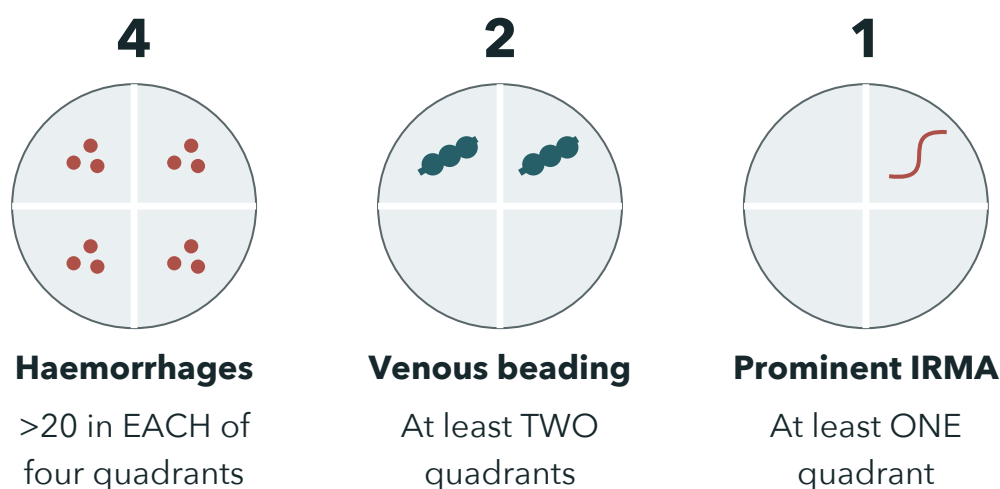
Do not grade from acuity

A person can read 6/6 while severe NPDR progresses. Examine the whole retina and assess macular oedema separately; preserved central vision does not make the peripheral microangiopathy mild.

References AAO: Diabetic Retinopathy PPP, 2025 | NICE NG 242: Diabetic retinopathy management

The 4-2-1 rule is a distribution test

Any one criterion can establish severe NPDR



Any ONE criterion + no new vessels = severe NPDR

Original distribution schematic. Confirm these findings on examination; this is not a patient image.

Turn the grade into a monitoring decision

- 1** Check for neovascularisation before using the NPDR label.
- 2** Document extent, macular status and systemic risk factors.
- 3** Arrange ophthalmic monitoring according to severity; progression, new vessels or macular oedema change the plan.

Common boundary error

Use venous beading in at least two quadrants, not more than two. IRMA remains within retinal tissue; neovascularisation grows on the retinal surface and crosses into the preretinal space.